Therapeutic Relationships, Abuse, Crisis, and Violence: One-Page Study Sheet

1. Therapeutic Nurse–Client Relationship & Boundaries (CNO & Varcarolis)

- **Therapeutic relationship**: Goal-oriented, time-limited, and professional; focused on the client's health and recovery, not meeting the nurse's personal needs.[1][2]
- **Phases**: Orientation (build trust, clarify roles and goals), Working (explore issues, promote change), Termination (review progress, plan for independence).[^2]
- **Core nurse qualities**: Empathy, respect, genuineness, and unconditional positive regard, expressed through active listening, attending behaviours, and cultural sensitivity.[3][2]
- **Boundaries**: Invisible limits that keep the relationship professional and prevent role confusion, favoritism, and exploitation.[4][5]
- **Boundary warning signs**: Secrecy, gifts, special favours, social media contact, outside-of-care meetings, and over-involvement or under-involvement with a client.[6][4]
- **Nurse responsibilities**: Establish and maintain boundaries, use a clear plan of care, explain the purpose and time limits of the relationship, maintain confidentiality, seek guidance, and end relationships appropriately.[5][1] [^4]

2. Protecting Clients from Abuse (CNO & Varcarolis)

- **Abuse definition**: Any behaviour in the nurse–client relationship that is violent, threatening, neglectful, humiliating, sexually inappropriate, or otherwise exploits the power imbalance.[4][5]
- **Forms of abuse**: Physical, sexual, emotional/psychological, verbal, financial abuse, and neglect.[7][4]
- **Absolute prohibitions**: Nurses must never engage in romantic or sexual relationships with current clients, and must not form sexual relationships with former clients for a set period after the relationship ends (e.g., at least one year).[^4]
- **Nurse duties**: Recognize early signs of abuse, take immediate steps to stop or prevent harm, ensure safety, document clearly, and report to employers, CNO, and other regulators as required.[5][7] [^4]
- **Therapeutic stance**: Believe the client, validate their experiences, avoid blaming, and work collaboratively on safety and support.[2][3]

3. Crisis and Disaster (Varcarolis Ch. 21)

- **Crisis**: A temporary state of psychological disequilibrium where usual coping strategies fail to resolve a stressor, leading to significant distress and impaired functioning.[8][9]
- **Types of crises**: Maturational (developmental transitions), Situational (unexpected events such as illness, job loss), Adventitious or social (disasters, community violence, terrorism).[10][8]
- **Determinants of outcome**: Person's perception of the event, social supports, and available coping skills/resources.[9][8]
- **Goals of crisis intervention**: Ensure physical safety, reduce anxiety, restore the person to pre-crisis level of functioning or better, and prevent long-term psychological harm.[11][8]
- **Key nursing actions**: Rapid assessment of risk (harm to self/others), simple and direct communication, problem-solving support, mobilizing social and community resources, and short-term, focused follow-up.[8][11]
- **Disaster context**: Nurses contribute to triage, psychological first aid, basic needs, and ongoing monitoring of trauma reactions in individuals and communities.[12][11]

4. Suicide and Non-Suicidal Self-Injury (Varcarolis Ch. 22)

- **Suicide risk factors**: History of attempts, mental disorders (especially mood and substance-use disorders), hopelessness, recent loss or crisis, chronic pain, isolation, and access to lethal means.[^13]
- **Warning signs**: Talking about wanting to die, seeking means, expressing hopelessness or feeling like a burden, sudden calmness after severe depression, or significant behaviour change.[^13]
- **Assessment tools**: Use structured approaches (e.g., IS PATH WARM-style questions) plus clinical judgment to explore intent, plan, means, and protective factors.[^13]
- **Prevention levels**: Primary (population-level education and stigma reduction), Secondary (immediate intervention in crisis), Tertiary/postvention (support after attempts or death to reduce further risk).[^13]
- **Non-suicidal self-injury (NSSI)**: Deliberate self-harm without intent to die, often used to regulate overwhelming emotions; requires validation, assessment of function, and skills-based interventions, not punishment.[^13]
- **Nursing priorities**: Ensure immediate safety, remove or reduce access to means, maintain close observation when indicated, develop collaborative safety plans, and use non-judgmental therapeutic communication.[^13]

5. Violence Across the Lifespan (Varcarolis Ch. 24 & Ontario/Peel Resources)

Child Abuse and Neglect

- **Legal duty**: Everyone in Ontario must immediately report any reasonable suspicion that a child is in need of protection to a children's aid society; proof is not required.[14][15]
- **Forms**: Physical, sexual, emotional abuse, neglect, and exposure to intimate partner violence.[16][14]
- **Reasonable grounds**: Information that an average person, using normal judgment, would consider sufficient to suspect a child may be at risk; new concerns must be reported each time.[15][14]
- **Nursing role**: Recognize indicators (injuries, developmental delays, fearfulness, inconsistent stories), document objectively, report promptly, and collaborate with protection agencies.[9][14]

Elder Abuse

- **Definition**: Any act or failure to act within a relationship of trust that causes harm or risk of harm to an older adult (at home, in the community, or in care settings).(Ontario.ca elder abuse)[^17]
- **Types**: Physical, emotional/psychological, sexual, financial abuse, and neglect (e.g., withholding care, food, or medications).[^17]
- **Warning signs**: Unexplained injuries, poor hygiene, sudden changes in finances or living arrangements, fearfulness, withdrawal, or a caregiver speaking for or controlling the senior.[^17]
- **Response**: Listen, believe, avoid blaming, assess immediate safety, encourage safety planning, and connect to resources (Seniors Safety Line, community agencies, police). Mandatory reporting rules apply in some care settings.[^17]

Intimate Partner and Family Violence (IPV/FIPV)

- **Definition**: Patterns of physical, sexual, emotional, or financial abuse, threats, and coercive control within an intimate or family relationship.[18][19]
- **Examples**: Insults and humiliation, isolation from supports, controlling money and decisions, threats to harm partner or children, destruction of property, using weapons or pets to intimidate.[19][18]
- **Health impacts**: Physical injuries, chronic pain, sexually transmitted infections, mental health problems (anxiety, depression, PTSD), substance use, and increased risk of homicide.[18][19]
- **Peel context**: Intimate partner violence has been identified as a serious and escalating issue, with specialized police units and the Safe Centre of Peel providing coordinated response and supports.[20][18]
- **Nursing responsibilities**: Routinely and sensitively screen when safe, ensure privacy, validate the person's experience, assess lethality risk, help develop safety plans, document accurately, know local resources, and follow legal/reporting requirements.[19][9][^18]

6. Quick Nursing Practice Checklist

- **Maintain professionalism**: Keep relationships time-limited, goal-oriented, and client-centred; monitor for boundary crossings and seek consultation early.[1][4]
- **Prioritize safety**: In all situations of crisis, suicidality, abuse, or violence, assess immediate risk and act to protect the client and others.[11][14] [^8]
- **Know legal duties**: Understand and follow mandatory reporting laws for child abuse, applicable rules for elder abuse and institutional settings, and professional obligations to report unsafe colleagues.[14][15] [^17]
- **Communicate therapeutically**: Listen actively, validate feelings, avoid victim-blaming, and use clear, simple, and compassionate language.[3][2] [^9]
- **Collaborate and refer**: Work with interprofessional teams (CAS, police, crisis lines, shelters, community agencies) and provide clients with information about resources and supports.[9][18] [^19]
- **Reflect on self**: Be aware of personal reactions, biases, and vicarious trauma; use supervision, debriefing, and self-care to support safe, ethical practice.[2][3]

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